Bevacizumab + weekly paclitaxel

National Cancer Control Programme (NCCP) 00738 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your treatment and access plan

Bevacizumab 10 mg/kg is given on Days 1 and 15, with paclitaxel 80 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Continue according to benefit, toxicity and the treating team's plan.

STEP 1

Before treatment

Baseline full blood count (FBC), kidney/liver tests and urine protein. Review blood counts, renal/liver function and urine protein before cycles; Day 8 FBC is explicitly required. Check current counts for each paclitaxel decision.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. A port is not automatically required; insertion and removal are separate procedures with individual instructions.

STEP 3

On the treatment day

Confirm the medicines due that day, any premedicines and the reviewed blood results. Tell staff immediately about pain, burning, rash or breathlessness.

STEP 4

After the visit

No routine take-home infusion pump is required for this regimen. Keep line-care, urgent-contact and next-review instructions.

Visit length

Bevacizumab: 90 minutes initially, then 60 and 30 minutes if tolerated; paclitaxel: about one hour. Full visit time is longer.

Line care

A PICC or port has its own care plan. Report fever, redness, swelling, pain, leakage or line damage; do not remove or adjust it yourself.

Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Support for this visit

Separate the first paclitaxel premedication from later weekly doses; review blood pressure, urine protein, wounds and surgery timing for bevacizumab.

Sources: NCCP 00738 v3 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.